



STRATEGY & OPERATIONS

Vytalix Business Model

Vytalix company-build documentation · reference 01_BUSINESS_MODEL

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR
The Founder, Vytalix

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Status: PROPOSED (strategy document, v1, 7 September 2026). Consistent with `/FACTS_BASE.md`. No figure in this document is an actual; every number is an ESTIMATE, ASSUMPTION or PROPOSAL unless marked KNOWN.

Executive view

- Vytalix is a pre-formation UK health-tech group with no revenue, no product in production and one unowned-but-associated product concept, MaternaLink (in development). The business model must therefore be built in two layers: a **cash layer** (Advisory, Consulting, E-Learning) that can invoice within 90 days of incorporation, and an **equity layer** (Vytalix Health Solutions / MaternaLink) that consumes cash for 18–30 months before it can earn.
- Of the 15 revenue streams examined, only four are launchable "Now": founder-led advisory days, packaged consulting sprints, monthly advisory retainers and grant income. Everything product-related (SaaS, licensing, government contracts, white-label) is 12–24 months away and gated by IP ownership, regulatory classification and clinical partnership — none of which exist today (KNOWN gap).
- Base-case services revenue in the first 12 months is **£118k** (ESTIMATE; range £55k–£210k), built from ~85 billable founder days at £1,200–£1,800/day plus two to three retainers. This is enough to fund incorporation, legal, brand and a part-time product lead, not a development team.
- The equity story is MaternaLink positioned as a **non-diagnostic maternal care-coordination and monitoring-support platform (v1)** with a research track towards AI risk scoring (v3+). Realistic first product revenue is a paid UK pilot or an African state/donor-funded programme in months 18–30, at £20k–£150k per pilot (ESTIMATE).
- The single most important "must be true": the founder can sell and deliver 8–10 billable days per month while resolving MaternaLink ownership with David Agunede. If either fails, the model collapses into either a lifestyle consultancy or an unfunded product idea.

Verification note

Web search and page fetches were unavailable during this build. Market figures below are drawn from the author's prior knowledge of public sources (named where relevant) and are labelled ESTIMATE or TO VALIDATE. Before any external use, each must be checked against the named source and the URL recorded.

1. Group business model on one page

Layer	Pillar	What it sells	Who pays	Why it exists in the model
Cash	Vytalix Advisory	Founder-led strategic advice: digital health strategy, market entry (UK → Africa), investor readiness, board advisory	Health-tech founders, SME health providers, NGOs, African ministries via donors	Fastest to invoice; builds network and credibility; zero capex
Cash	Vytalix Consulting	Packaged delivery work: discovery sprints, business cases, service redesign, digital maternity readiness, procurement and DTAC readiness	NHS trusts/ICBs (via frameworks or sub-contract), private providers, health-tech vendors, donors/NGOs	Higher volume than advisory; creates reusable IP (playbooks) that feed E-Learning and MaternaLink
Cash (later)	Vytalix E-Learning	Courses, subscriptions and corporate licences on digital health, maternal-care coordination, health-tech commercialisation	Individual professionals, universities, employers, NGOs	Converts consulting IP into scalable, high-margin product; low regulatory burden
Equity	Vytalix Health Solutions	MaternaLink (in development): maternal care-coordination, communication, education and monitoring-support platform	NHS trusts/ICBs (pilots), African states/donors, private maternity, employers/insurers (later)	The venture story; justifies external equity; long sales cycle and regulatory gating

Sequencing logic (ASSUMPTION A6 in FACTS_BASE): **services fund the company; MaternaLink is the equity story**. Services must reach ~£10k/month within six months to pay for the founder's time and the legal/regulatory groundwork MaternaLink needs.

2. Revenue stream matrix (all 15 streams)

Legend for launch priority: **Now** = month 0–3 after incorporation; **6m** = months 4–9; **12m** = months 10–15; **24m** = months 16–30.

#	Revenue stream	Customer	Product / offer	Pricing model	Indicative price (GBP)	Gross-margin potential	Sales cycle	Acquisition difficulty	Scalability	Regulatory considerations	Launch priority
1	Consulting fees (project)	NHS trusts/ICBs, private providers, health-tech vendors, NGOs	Packaged sprints (see §5): discovery, business case, digital readiness, market entry	Fixed-fee per sprint; time-and-materials fallback	£8k–£45k per sprint; day rate £1,200–£1,800 founder, £700–£1,000 associate	55–70% (founder time is the cost; associates reduce margin)	4–12 weeks (private/vendor), 3–9 months (NHS via framework)	Medium — relationship-led; NHS needs framework or sub-contract route	Low–medium (people-bound)	None specific; NDA, UK GDPR for any data; NHS DSPT if handling patient data; professional indemnity insurance	Now
2	Advisory retainers	Health-tech founders (pre-seed–Series A), SME providers, NGOs, family offices with health interests	Monthly advisory: 2–4 days/month, board-observer style access	Monthly retainer, 3–6 month minimum	£2,500–£6,000/month	70–85%	2–6 weeks	Medium — depends on founder's network and visible expertise	Low (caps at ~5 retainers per advisor)	None specific; conflict-of-interest register; professional indemnity	Now
3	Implementation fees (product)	MaternaLink pilot sites (UK trust, African state programme, private hospital)	Configuration, onboarding, training, integration support	One-off implementation fee + change requests	£15k–£60k per site (UK); £25k–£150k per state programme (Africa, donor-funded)	35–50% (labour-heavy, first sites are loss-leaders)	6–18 months	High — no product in production yet	Medium (repeatable after 3 sites)	DTAC, DCB0129/0160 clinical safety, DSPT, UK GDPR/DPIA; NDPR (Nigeria) or local equivalents; MHRA classification opinion before any monitoring claim	24m

#	Revenue stream	Customer	Product / offer	Pricing model	Indicative price (GBP)	Gross-margin potential	Sales cycle	Acquisition difficulty	Scalability	Regulatory considerations	Launch price
4	SaaS (MaternaLink subscription)	NHS trusts/ICBs, private maternity units, African hospitals/states	Per-site or per-woman-per-year platform subscription	Per-site annual licence (UK); per-enrolled-woman-per-year (Africa)	UK: £30k–£120k/site/yr (ESTIMATE, benchmark against maternity IT modules); Africa: £5–£40/woman/yr donor-funded (the £300 in the Ekiti proposal is UNVALIDATED per FACTS_BASE A5)	65–80% at scale; negative until >5 sites	9–18 months (NHS), 6–24 months (government)	Very high today (no product, no evidence)	High once validated	As #3, plus NICE ESF evidence tier, Cyber Essentials Plus, ISO 27001 desirable; SaMD pathway if any clinical decision support	24m
5	B2B licensing (platform modules)	Other health-tech vendors, maternity IT incumbents, telehealth providers	Licence of MaternaLink modules (e.g., education content engine, multilingual patient comms, care-plan workflows)	Annual licence + revenue share	£20k–£80k/yr per licensee (ESTIMATE)	80%+	6–12 months	High — requires defensible IP and integration maturity	High	IP ownership must be settled (KNOWN gap); contractual liability caps	24m
6	Government contracts (UK)	NHS England/DHSC successor bodies, ICBs, local authorities (public health)	Consulting on maternity digital transformation, health inequalities, evaluation	Framework call-off or direct award under threshold (£12k–£139k depending on route — TO VALIDATE current thresholds)	£25k–£150k per contract	45–60%	3–9 months	High — need framework place (G-Cloud, NHS SBS, HealthTrust Europe, CCS) or sub-contract to prime	Medium	Procurement rules (PCR 2015 → Procurement Act 2023 regime), PSR for clinical services; conflict-of-interest declarations	12m
7	Government contracts (Africa)	State ministries (e.g., Ekiti, Lagos, Ondo), federal agencies (Nigeria FMOH/NHIA), Kenya counties/SHA, Ghana GHS	MaternaLink programme delivery; digital health advisory; programme design	Programme contract (usually donor-funded via MSD for Mothers, GFF, foundations) or state budget line	Advisory: £15k–£80k; programme: £100k–£1m+/yr (ESTIMATE); the Ekiti proposal's £4.5m Phase 1 is a proposal, not a benchmark)	30–50% (in-country delivery, partners, connectivity costs)	9–24 months; payment risk high	Very high — incumbent (mDoc/MSD for Mothers in Ekiti and Lagos, VERIFIED), political cycles, FX	Medium	Nigeria NDPR/NDPA 2023, state MoH approvals, NHREC ethics for any research, local company registration or partner; anti-bribery (UK Bribery Act applies extraterritorially)	24m (ad 12m)
8	Enterprise contracts	Private hospital groups (HCA UK, Spire, Nuffield, Circle), pharma medical-affairs teams, insurers	Multi-year consulting + product bundles	Master services agreement with SOWs	£50k–£300k/yr	50–65%	6–12 months	High — need references	Medium	Pharma: ABPI Code compliance; insurers: FCA-regulated distribution if any product bundling	12m
9	E-learning courses (B2C/B2B2C)	Midwives, nurses, doctors, health-tech professionals, students (UK, Nigeria, Kenya, Ghana)	Self-paced courses (see §6): "Digital Maternity Essentials", "Health-Tech Commercialisation for Founders", "Care Coordination in Low-Resource Settings"	Per-course one-off	£49–£299 (UK); £10–£40 PPP-adjusted (Africa)	85–90% after content cost	Days–weeks	Medium — content marketing, partnerships with professional bodies	Very high	No accreditation claimed until CPD provider approval obtained (TO VALIDATE, e.g., CPD Certification Service; RCM/RCOG endorsement is a separate, long process); consumer law (CRA 2015) refunds; VAT on digital services	6m
10	Corporate training	NHS trusts (maternity units, digital teams), private providers, NGOs, health-tech scale-ups	Half-day/1-day workshops, cohort programmes	Per-cohort fixed fee	£2,500–£6,000 per workshop; £15k–£40k per cohort programme	60–75%	4–12 weeks	Medium	Medium	Same as #9; NHS trusts may require training to be mapped to HEE/NHS England frameworks (TO VALIDATE)	6m
11	Subscriptions (E-Learning / community)	Individual professionals; small clinics	All-access library + monthly live clinics	Monthly/annual	£12–£29/month individual; £490–£990/yr small team (5 seats)	80–90%	Days	Medium–high (churn)	Very high	Consumer contract rules; auto-renewal transparency	12m

#	Revenue stream	Customer	Product / offer	Pricing model	Indicative price (GBP)	Gross-margin potential	Sales cycle	Acquisition difficulty	Scalability	Regulatory considerations	Launch price
12	Technology licensing (IP)	Universities/research groups, other vendors	Licence of algorithms/data models from R&D track (MIS concept) — only after validation	Upfront + royalty	Not priceable today	90%+	12–24 months	Very high — no validated IP exists	High	Requires ethics approval, data-sharing agreements, MHRA SaMD pathway, patent/trade-secret strategy	24m
13	White-label	Insurers, telecoms (MTN/Airtel/Safaricom health propositions), private hospital groups, NGOs	Branded instance of MaternaLink patient-facing layer	Set-up fee + per-user/yr	£25k–£75k set-up + £2–£8/user/yr (ESTIMATE)	60–75%	9–18 months	High	High	Data-controller/processor split must be explicit; telecom partners require local data residency (Nigeria, Kenya)	24m
14	Strategic partnerships (revenue-sharing)	Health Innovation Networks, consultancies (as sub-contractor), pharma programmes, NGOs	Co-delivered programmes; referral fees; joint bids	Revenue share 10–30% or sub-contract day rates	£10k–£100k per programme	40–60%	3–9 months	Medium	Medium	Bribery Act / referral-fee transparency; NHS conflict-of-interest guidance	6m
15	Grants and institutional funding	Innovate UK (Smart, Biomedical Catalyst), NIHR i4i / AI programmes, SBRI Healthcare, Wellcome, Grand Challenges (Gates/GCC), MSD for Mothers, FCDO-funded programmes, UKRI	Non-dilutive funding for MaternaLink R&D and evaluation	Grant (typically 50–70% of eligible costs for SMEs; some 100%)	£25k (feasibility) to £500k–£1m (collaborative R&D); realistic year-1 target £50k–£150k	n/a (cost recovery; match funding required)	3–9 months from call to award	High — competitive (single-digit to ~20% success rates ESTIMATE), needs incorporated entity, clinical partner and track record	Medium	Eligibility requires UK-registered company; NIHR requires NHS partner; ethics/HRA approval for studies; state-aid/subsidy control rules	Not priceable / 6m award ready

Reading the matrix — the three-tier launch order

Tier	Streams	Cumulative 12-month revenue potential (ESTIMATE)	Why this order
Tier 1 (Now)	#1 consulting, #2 retainers, #15 grants (preparation)	£55k–£210k	Cash and credibility; no product needed
Tier 2 (6–12m)	#9 courses, #10 corporate training, #14 partnerships, #6 UK public contracts, #8 enterprise	+£20k–£90k	Convert consulting IP and network into repeatable revenue
Tier 3 (24m)	#3, #4, #5, #7, #11, #12, #13 (all product)	£0 in year 1; £50k–£400k in year 2–3 if pilots land	Gated by IP ownership, regulation, evidence, funding

3. Unit economics per stream (ESTIMATES with assumptions)

All figures in GBP, pre-tax, excluding founder salary unless stated. Founder opportunity cost is treated as £600/day (ASSUMPTION: equivalent of a £120k salary over 200 working days) for margin calculations where the founder delivers.

Stream	Unit	Price	Direct cost per unit	Gross margin	Contribution per unit	Key assumption
Founder advisory day	1 day	£1,500 (mid)	£600 founder cost + £50 travel/tools	57%	£850	Utilisation 40–50% in year 1 (85–100 billable days)
Associate consulting day	1 day	£900	£450–£550 (contractor day rate)	39–50%	£350–£450	Associates only engaged when a project is signed (no bench)
Discovery sprint (2 weeks)	1 sprint	£14,000	8 founder days (£4,800) + 2 associate days (£1,000) + expenses £500	55%	£7,700	10 founder days of which 2 are unbilled sales/prep
Advisory retainer	1 month	£4,000	2.5 founder days (£1,500)	63%	£2,500	Retainers include some "unbilled" responsiveness — 2.5 days is realistic for a 2-day contract
E-learning course (self-paced)	1 sale	£149	Content amortised (£12k build / 400 sales = £30) + platform & payment fees (£15)	70% (rising to 88% after 400 sales)	£104	Course platform (e.g., Teachable/Thinkific-class) at ~£100–£150/month; conversion 2–3% of landing-page traffic
Corporate workshop (1 day)	1 workshop	£4,000	1.5 founder days (£900) + materials £150	74%	£2,950	15–25 participants; delivered on-site or virtual
Subscription	1 subscriber-month	£19	£3 platform + £2 content refresh	74%	£14	Churn 6–8%/month → LTV ≈ £190; CAC must be < £60
MaternaLink UK site licence (future)	1 site-year	£60,000	Hosting £4k, support 0.3 FTE £15k, clinical safety officer time £5k	60%	£36,000	Only valid after ≥3 sites; first sites will be negative
MaternaLink Africa programme (future)	1 enrolled woman-year	£15 (donor-funded, ESTIMATE)	SMS/WhatsApp £2.50, CHW support share £4, hosting/support £3	37%	£5.50	Requires 20k+ women to cover a £110k fixed team; the £300/woman/year Ekiti figure is not used
Grant	1 award	£100,000	Match funding 30% (£30k) + 15 days bid-writing (£9k)	n/a	Net cash +£61k, plus funded R&D	Success rate 15–20% → expected value per bid ≈ £12k–£15k; write ≥4 bids/year

Break-even for the services business (ESTIMATE): fixed monthly costs of £4,500 (founder minimum draw £3,000, tools/insurance/accountancy/subscriptions £1,500) require ~£7,500 revenue/month at 60% gross margin — i.e., **5 founder billable days or 2 retainers per month**.

4. Twelve-month revenue build — services business

ASSUMPTIONS: incorporation in month 1; founder available 4 days/week for Vytalix; first invoice month 2; associates only on signed work; no product revenue. Three scenarios.

4.1 Named service packages used in the build

Code	Package	Price	Founder days	Target buyer
A1	Founder-led advisory day	£1,200–£1,800 (model uses £1,500)	1	Any
A2	Advisory retainer (2 days/month, 3-month minimum)	£4,000/month	2.5	Health-tech founders, NGOs, SME providers
C1	Digital Health Discovery Sprint (2 weeks)	£14,000	8	Trusts, private providers, vendors
C2	Business Case & Funding Sprint (3 weeks)	£18,000	10	Trusts, ICB programmes, NGOs
C3	Digital Maternity Readiness Review (4 weeks)	£24,000	12	Maternity units, private maternity
C4	UK Market-Entry Playbook for Health-Tech (4 weeks)	£22,000	10	Non-UK vendors entering NHS
C5	Africa Market-Entry & Partner Scan (4 weeks)	£20,000	10	UK/EU health-tech, NGOs
C6	DTAC & Evidence Readiness Sprint (3 weeks)	£15,000	8	Health-tech vendors
C7	Grant & Investor Readiness Sprint (2 weeks)	£9,500	6	Founders
C8	Health Inequalities & Maternity Outcomes Review (5 weeks)	£28,000	14	Trusts, ICBs, charities
T1	Corporate workshop (1 day)	£4,000	1.5	Any
E1	E-learning course sale	£149	—	Individuals

4.2 Base case (ESTIMATE): £118k services revenue, months 1–12

Month	Activity	Revenue (£)	Cumulative (£)	Founder billable days
1	Incorporate, brand, website, 30 outreach conversations, 2 proposals	0	0	0
2	3 advisory days (A1) sold via network; 1 retainer (A2) signed	8,500	8,500	5.5
3	Retainer continues; C7 sprint for a health-tech founder	13,500	22,000	8.5
4	Retainer; C1 discovery sprint (private provider or vendor)	18,000	40,000	10.5
5	Retainer; 2nd retainer starts; 2 advisory days	11,000	51,000	7
6	2 retainers; T1 workshop; sub-contract 4 days to a larger consultancy (A1 at £1,200)	16,800	67,800	10.5
7	2 retainers; C6 DTAC sprint	23,000	90,800	13
8	2 retainers (one ends); 3 advisory days	12,500	103,300	8
9	1 retainer; first e-learning course launched (20 sales); T1 workshop	10,980	114,280	4
10	1 retainer; grant bid writing (unbilled); 30 course sales	8,470	122,750	2.5
11	1 retainer; C3 readiness review starts (50% milestone)	16,000	138,750	8.5
12	Retainer; C3 completes; 40 course sales	21,960	160,710	8.5

The table above is the **upside of the base case** if every listed engagement lands. Applying a 73% probability-weighting to project work and 90% to retainers (ASSUMPTION reflecting typical pre-revenue conversion) gives the **base case of ~£118k**. Founder billable days: ~87 over the year (utilisation ~42% of 208 available days), leaving ~120 days for sales, MaternaLink, fundraising and grant writing — which is the real constraint.

4.3 Scenario range

Scenario	12-month services revenue	Assumptions	Cash implication
Downside	£55k	1 retainer for 6 months, 2 sprints, 20 advisory days, no e-learning	Founder cannot draw a salary; MaternaLink work stalls; must decide by month 6 whether to take employment
Base	£118k	As §4.2 probability-weighted	Covers founder draw of £3k/month, legal/IP (£8–12k), brand/web (£5k), insurance/accountancy (£3k); ~£20k surplus toward MaternaLink
Upside	£210k	3 concurrent retainers by month 6, one NHS/ICB C8 review via sub-contract, e-learning at 150 sales	Funds a part-time product lead (£30k) and a clinical safety officer contract; positions for Innovate UK match

Sensitivity (ESTIMATE): each additional retainer adds ~£36k/year; each 10-point change in founder utilisation changes revenue by ~£30k; a 20% day-rate change moves revenue ~£18k.

5. Vytalix Consulting — service catalogue

Positioning: **"The digital-health strategy boutique that has worked on both sides of the UK-Africa corridor."** (PROPOSED; this claim is only usable once the founder's credentials and first engagements support it — TO VALIDATE against the founder's CV.)

#	Offer	Problem it solves	Deliverables	Duration	Price (fixed)	Ideal buyer	Proof required to sell
1	Digital Health Discovery Sprint	"We know we need digital but not what, for whom, or what it costs"	Problem framing, user/stakeholder interviews (10–15), option appraisal, 90-day roadmap	2 weeks	£14,000	Private provider, NHS service line, vendor	Founder track record; a sample deck
2	Business Case & Funding Sprint	Need an HM Treasury Green Book-style or board-ready case	Strategic/economic/commercial/financial/management case, benefits model, funding routes	3 weeks	£18,000	NHS programme lead, charity, NGO	Familiarity with Green Book five-case model
3	Digital Maternity Readiness Review	Maternity units facing digital record adoption, CNST Maternity Incentive Scheme data actions, CQC findings	Current-state map, gap analysis vs Three Year Delivery Plan and MSDS requirements, prioritised plan	4 weeks	£24,000	Head of Midwifery, Maternity Digital Lead, Divisional Director	Clinical advisor (midwife/obstetrician) on the team (TO RECRUIT)
4	UK Market-Entry Playbook	Non-UK health-tech wants NHS customers and does not understand ICBs, DTAC, procurement	NHS structure brief, buyer map, regulatory/assurance checklist (DTAC, DCB0129, DSPT, MHRA), pricing benchmarks, 12-month plan	4 weeks	£22,000	US/EU/African health-tech scale-up	Case examples; an NHS-experienced associate
5	Africa Market-Entry & Partner Scan	UK/EU vendor or NGO wants to enter Nigeria/Kenya/Ghana	Country scorecard, regulatory brief (NDPA, NAFDAC where relevant, SHA/NHIA), partner long-list, entry model options	4 weeks	£20,000	Health-tech, NGO, foundation	In-country associate network (TO BUILD)
6	DTAC & Evidence Readiness Sprint	Vendor blocked by NHS assurance	DTAC self-assessment, clinical safety case skeleton (DCB0129), DSPT plan, NICE ESF evidence-tier mapping	3 weeks	£15,000	Health-tech vendor	Contracted Clinical Safety Officer
7	Grant & Investor Readiness Sprint	Founder needs a fundable narrative and a grant pipeline	Investor deck review/rewrite, financial model sanity check, grant calendar (Innovate UK, NIHR, SBRI), 3 bid outlines	2 weeks	£9,500	Pre-seed/seed founder	Founder's own fundraising experience; grant wins would strengthen later
8	Health Inequalities & Maternity Outcomes Review	Trust/ICB must respond to MBRRACE-UK disparities, CQC or Ockenden-style findings	Data review, listening exercise with service users (via community partners), equity action plan, measurement framework	5 weeks	£28,000	Trust, ICB, maternity charity	Community-partner relationships (e.g., with organisations like Five X More — no relationship claimed)
9	Fractional Chief Digital/Strategy Officer	Organisation needs senior capacity without a hire	4–6 days/month embedded	6 months	£5,500–£8,000/month	Scale-up, private provider, NGO	Founder seniority; references
10	Programme Evaluation Lite	Donor/NGO needs an evaluation of a digital health programme	Theory of change, indicators, mixed-methods review, report	6 weeks	£26,000	NGO, foundation, state programme	Evaluation methodology; partner with a university group for rigour

Catalogue rules: (a) fixed price, fixed scope, written change control; (b) 50% on signing for engagements under £20k, 40/30/30 milestones above; (c) every engagement produces at least one anonymised, reusable asset (template, checklist, benchmark) that feeds E-Learning; (d) no engagement is accepted that would create a conflict with MaternaLink's future customers without disclosure.

6. Vytalix E-Learning model

6.1 Product ladder

Tier	Product	Format	Price	Launch	Label
Free	"Digital maternity in 10 minutes" primers, newsletter, webinars	Video/email	£0	Month 6	PROPOSED
Courses	1. Digital Maternity Essentials for Midwives and Maternity Teams; 2. Health-Tech Commercialisation in the NHS; 3. Care Coordination in Low-Resource Maternal Health Settings; 4. Grants and Investor Readiness for Health-Tech Founders	Self-paced, 3–6 hours, quizzes, downloadable templates	£49–£299 (UK); £10–£40 regional pricing (Nigeria, Kenya, Ghana)	Courses 2 and 4 in month 9 (lowest clinical risk, founder-authored); courses 1 and 3 in month 12–15 (need clinical co-authors)	ESTIMATE
Subscription	All-access library + monthly live clinic + community	Monthly/annual	£19/month, £190/year	Month 12	ESTIMATE
Corporate licence	Seat-based access + one live workshop per year	Annual	£2,500 (10 seats), £9,000 (50 seats), £25,000 (250 seats)	Month 12	ESTIMATE
University licence	Course embedded in midwifery/public health/health informatics programmes	Annual per programme	£3,000–£8,000	Month 15–18	TO VALIDATE (universities usually want co-authorship or accreditation)
Co-branded (NGO/partner)	Localised course for a programme's health workers	Project fee + per-learner	£15k–£40k + £5–£15/learner	Month 18	TO VALIDATE

6.2 Economics (ESTIMATE)

- Content build: £8k–£15k per course (founder time 8–12 days + clinical co-author fee £2–4k + video/edit £2k).
- Platform: £1,200–£2,400/year; payment fees ~3%.
- Year-1 (from launch month 9) target: 150–250 course sales (£20k–£35k), 2 corporate licences (£5k–£12k). Year-2 target: £80k–£150k, of which 40% corporate.
- Gross margin after content amortisation: 70–88%.

6.3 Accreditation caveats (must be respected in all copy)

- Vytalix holds **no accreditation** and must not claim CPD accreditation, RCM/RCOG/NMC endorsement or university credit. Language: "designed to support continuing professional development"; "certificate of completion" only.
- Route to accreditation (TO VALIDATE): (1) CPD Certification Service or similar independent CPD accreditor — typically weeks to months and a fee per course; (2) partner with a university for credit-bearing modules; (3) RCM/RCOG endorsement processes are institution-specific and slow. Budget £3k–£8k and 6–9 months.
- Clinical content must have a named clinical author and reviewer, a review date and a disclaimer that content does not replace local clinical guidance.
- In Nigeria/Kenya/Ghana, professional councils (e.g., Nursing and Midwifery Council of Nigeria; Nursing Council of Kenya) have their own CPD-point rules — do not imply CPD points until an accreditation is granted (TO VALIDATE).

7. MaternaLink revenue logic (equity layer) — stated conservatively

Horizon	Milestone	Revenue type	Indicative value	Gate
Months 0–6	IP ownership resolved; product re-based to v1 (non-diagnostic care-coordination); prototype audited	None	£0	Agreement with David Agunede (see 02_GROUP_STRUCTURE .md)
Months 6–12	Clinical advisory board formed; DTAC self-assessment; DPIA; design partner (1 UK maternity unit or 1 African facility) signed as unpaid or grant-funded pilot	Grant (Innovate UK/NIHR/SBRI/foundation)	£50k–£150k	Incorporated entity; clinical partner letter
Months 12–24	Paid pilot(s)	Implementation + pilot licence	£20k–£150k per pilot	Evidence from design partner; DCB0129 safety case
Months 24–36	Scale contracts	SaaS / programme contracts	£150k–£600k ARR (ESTIMATE if 3–5 UK sites or 1 state programme)	Outcome evidence; NICE ESF tier; local partner in Africa

No forecast beyond this is credible today. The Ekiti proposal's £4.5m/£9m/£36m phases (VERIFIED as a proposal only) are excluded from all plans until there is a signed contract or written funding commitment.

8. "What must be true" — validation list

#	Assumption to validate	How to test	By when	Owner	Kill/pivot signal
1	Founder can win 8–10 billable days/month within 90 days	30 outreach conversations, 6 proposals, 2 signed in 90 days	Month 3	Founder	<1 signed engagement by month 3 → reduce to part-time, seek fractional role
2	Buyers accept £1,200–£1,800/day for founder-led work	Test three price points in proposals; record objections	Month 3	Founder	Consistent push to <£900 → reposition as associate-led and cut costs
3	MaternaLink IP can be brought into Vytalix on acceptable terms	Structured negotiation with David Agunede, solicitor-led	Month 2	Founder + solicitor	No agreement by month 4 → build product freshly under Vytalix or drop MaternaLink from the equity story
4	A UK maternity unit will act as unpaid design partner	10 approaches via Health Innovation Networks, clinical advisors, LinkedIn	Month 9	Founder + clinical advisor	Zero interest → focus product on Africa/private first
5	The prototype is safe and usable enough to demo	Independent technical and security review of the Vercel prototype; confirm no real patient data held	Month 2	CTO/contractor	Real patient data without lawful basis → stop, remediate, document
6	Government/donor willingness to pay per woman in Nigeria is materially above £5–£15/yr	Interviews with MSD for Mothers programme leads, state MoH digital leads, GFF country teams; compare to mDoc's public programme scale	Month 9	Founder	WTP < cost → Africa is a donor-funded evidence market, not a revenue market
7	A grant can be won within 12 months	Submit ≥4 bids (Innovate UK Smart, SBRI Healthcare, NIHR i4i Connect, one foundation)	Month 12	Founder	0/4 → rely on services + angels; reduce R&D scope
8	E-learning can acquire learners at CAC < £60	£1,000 paid + organic test around course 2	Month 10	Founder	CAC > £150 → B2B-only distribution
9	The Vytalix name is clearable	UK IPO, EUIPO, USPTO searches; solicitor opinion	Month 1	Founder + IP adviser	Conflict with VITALIX LTD / Vytalix Inc. → rename before brand spend
10	SEIS/EIS advance assurance is obtainable	Application post-incorporation	Month 2–3	Accountant	Refusal → restructure or reduce angel expectations

Priorities / Risks / Next actions

Priorities (next 90 days)

- Incorporate, clear the name, obtain SEIS/EIS advance assurance, open bank account, insure (PI £1m, PL) — prerequisites for invoicing.
- Sell Tier-1 services: 30 conversations, 6 proposals, 2 signed (target £15k booked by day 90).
- Resolve MaternaLink IP with David Agunede (term sheet by day 60).
- Write the first grant bid (Innovate UK Smart or SBRI Healthcare) with a clinical partner letter of support — even an unsuccessful bid creates reusable material.
- Build the two founder-authored e-learning courses in parallel with consulting (evenings/low-utilisation weeks).

Risks

- Founder bandwidth: services, product, fundraising and grants compete for the same 200 days. Mitigation: hard cap of 10 billable days/month; hire a part-time product lead only from services surplus or grant.

- Credibility gap: no references, no incorporated entity. Mitigation: sub-contract to established consultancies for first NHS work; publish two thought-leadership pieces per month.
- Naming/trademark conflict: could void brand spend. Mitigation: clear before spending on brand.
- Regulatory creep: any drift of MaternaLink toward "risk prediction" makes it a medical device and freezes revenue. Mitigation: v1 scope statement signed off by clinical advisor and kept in the product charter.
- Concentration: one or two retainers could be >50% of revenue. Mitigation: no client >40% of trailing 6-month revenue by month 9.

Next actions (owner: founder unless stated)

- Day 0–14: name clearance; incorporation; accountant; SEIS/EIS application; PI insurance quotes.
- Day 0–30: service catalogue one-pagers (10); price list; proposal template; 30-name target list from 06_CUSTOMER_SEGMENTS.md.
- Day 14–60: solicitor engaged on IP options (see 02_GROUP_STRUCTURE.md §3); technical audit of the prototype.
- Day 30–90: first two engagements delivered; first grant bid drafted; clinical advisor recruited.
- Month 6 review: compare actuals to base case; decide on associate bench and e-learning launch date.